GLYCEMIC CONTROL



Studies have reported an association between hyperglycemia and adverse infectious and cardiovascular outcomes after cardiac and noncardiac surgery. [1-5] Many studies recommend delaying elective surgery until HbA1c levels are below 8.5, recognizing that this may not be feasible for all patients and that such a cutoff is not currently supported by robust evidence. [6-10]

Screening Tools

Glycemic Control Screening Questions extrapolated from BC Guidelines for Diabetes Care [11]

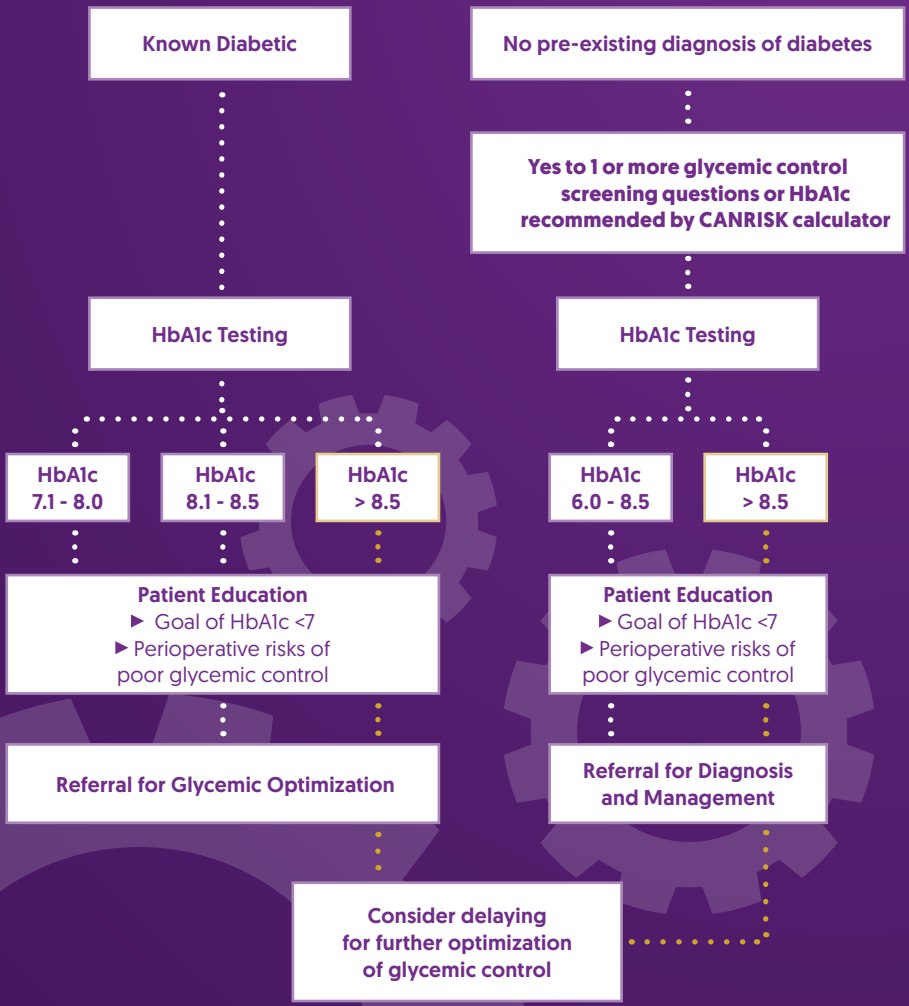
The CANRISK questionnaire is a validated risk tool for patients to self-assess their diabetes risk. [11]

Glycemic Control Screening Questions

	YES	NO
Previously diagnosed with diabetes	☐	☐
Age over 40	☐	☐
Family history of diabetes	☐	☐
History of hypertension, dyslipidemia or steroid use	☐	☐
BMI >25	☐	☐
History of gestational diabetes mellitus	☐	☐
History of pre-diabetes	☐	☐

SCORING If **YES** to any of the above questions then a preoperative screening hemoglobin A1c test is recommended.

GLYCEMIC CONTROL ALGORITHM



Prehabilitation and Optimization Recommendations

Patient Education	⚙ Reinforce general goal of A1c less than 7 [12] ⚙ Explain perioperative risks of hyperglycemia ⚙ Refer to online patient resources ⚙ If no previous diabetes diagnosis: • HbA1c 6.0-6.4: likely prediabetic • HbA1c > 6.5: likely diabetic
Referral for Diabetes Diagnoses and Management (no pre-existing diabetes)	⚙ Encourage follow-up with primary care provider for confirmation of diabetes diagnosis and initiation of management ⚙ Refer to online provider resources for referral templates
Referral for Glycemic Optimization (known diabetes)	⚙ Request re-evaluation from physician directing diabetes care with the goal of improving blood glucose control prior to surgery, if feasible ⚙ If holding GLP-1 agonist for a prolonged duration, adding other antihyperglycemic medication may be required ⚙ Refer to online provider resources for telehealth consultation options and referral templates
Consider Delaying for Glycemic Optimization	⚙ Discuss concerns, including potential benefits and risk of delaying surgery, with surgical team, particularly for total joint arthroplasty, vascular surgery, and spine surgery with instrumentation ⚙ Refer to local diabetes expert or online provider resources for telehealth consultation options